



A SOOTHEMADE BOOK

The Pregnancy Safety Reference

*The questions, organized.
The answers belong to your provider.*

Soothemade

A SMALL APOTHECARY · MADE SLOWLY

Dedication

For the pregnant person at 11pm on a Tuesday, googling whether deli meat is okay, getting six different verdicts, and going to bed unsettled.

This book is the bridge to a real conversation.

A note from Maya

Pregnancy is a long stretch of small questions.

Can I have deli meat. Can I drink coffee. Is hot yoga okay. Can I dye my hair. What about cleaning the bathtub. Is this medication safe. Can I travel. Can I lift this. Can I sleep on my back.

Most of these are five-second conversations with a provider. But the questions don't arrive when the provider is in the room. They arrive at 11pm on a Tuesday, when the only available answer is Google. Google's answer is six different forum posts with six different verdicts.

This book is the bridge. It organizes the categories of questions pregnant people commonly have, so you can carry them to your provider as a list, in one visit, instead of as a dozen 2am internet searches.

It does not answer your specific questions. Your provider does that. It is the question surface.

With care, Maya

This book is a question-organization reference, not medical advice. For specific safety questions about your pregnancy, please follow your obstetric provider's guidance. This book does NOT name medications. It does NOT make safety verdicts. It does NOT specify thresholds. The book is the list. The answers belong to your provider.

Chapter 1: Why This Book Doesn't Give Verdicts

This book does not say "yes" or "no" to your specific questions. Here is why.

Pregnancy safety is individual. "Is X safe in pregnancy" almost always has a more accurate answer than yes or no. The more accurate answer is "for whom, at what amount, at what stage, with what other factors." That sentence is a provider's sentence, not a reference book's.

The science updates. What was considered safe in one decade is sometimes revised. The current best advice for your country, your year, and your situation comes from a clinician who follows the current guidelines.

Risk tolerance varies. Some pregnant people want to be extra cautious. Some are okay with a wider margin. Both are reasonable. The right margin is the one you decide with your provider.

The book's only job: to get you to the conversation. Once you're in the conversation, you and the provider decide together.

A small principle: the internet's biggest weakness in pregnancy advice is that it cannot see YOU. It cannot see your medical history, your specific pregnancy, your particular risk factors. Your provider can. The book is here to get you to the provider faster.

The chapters that follow are eight categories of questions most pregnant people have. They are not exhaustive. They are starting points. If something is not in the book and is on your mind, write it down anyway. Bring it.

Chapter 2: Food

The category that gets googled the most.

The common subcategories below cover the foods most people have questions about. The point of this chapter is not to tell you which foods to avoid. The point is to give you the categories so you can bring them up with your provider in one focused conversation.

Deli and pre-made meats. Cold cuts at the store. Hot dogs and sausages. Pre-packaged deli sandwiches. Meat trays at parties.

Soft and unpasteurized cheeses. Brie, camembert, blue cheese, feta. Queso fresco, queso blanco. Anything "raw milk" or "unpasteurized."

Undercooked or raw protein. Sushi (raw fish vs cooked). Steak temperature. Runny eggs (poached, soft-boiled, over easy). Cookie dough. Caesar dressing (if raw egg). Tartare, carpaccio, ceviche.

Fish specifically. High-mercury vs low-mercury species. How much per week (your provider has the latest guidance). Smoked fish (lox, smoked salmon). Canned tuna.

Unpasteurized dairy and juice. Raw-milk products. Fresh-squeezed juice from a place that does not pasteurize. Farmer's market dairy.

Food handling and leftovers. How long can leftovers safely be kept. Reheating from frozen. Restaurant takeout left in the car. Buffets and salad bars.

Caffeine. Coffee daily limit. Caffeinated tea. Energy drinks, sodas. Caffeine in chocolate.

Sugar substitutes. Common artificial sweeteners. Stevia. Sugar alcohols.

Cultural or regional foods. Foods specific to your background that are not in standard guidance lists. Traditional or family recipes you grew up with. Bring the specific foods to your provider, especially if they are not common in their patient population.

Chapter 3: Drinks

Alcohol. The general guidance in most countries is zero alcohol during pregnancy. Your provider will be specific to you. If you drank before you knew you were pregnant, that is also a conversation to have.

Subcategories worth mentioning: cooking with alcohol (whether it cooks off), communion wine, non-alcoholic beer and wine.

Caffeine. See the food chapter. The conversation is the same.

Herbal teas. Common herbal teas (some are considered fine, some not, your provider will know which). Pregnancy-marketed teas (e.g. raspberry leaf). Detox teas. Kombucha.

Hydration generally. How much water per day in pregnancy. Electrolyte drinks (sports drinks). Coconut water.

Chapter 4: Activities

Exercise. Continuing your current exercise routine. Running while pregnant. Weight lifting. High-intensity classes. Pregnancy-modified options. Heart-rate caps (most current guidance has moved away from a specific number, but ask your provider).

Sex. When sex is and is not a question. Position considerations later in pregnancy. If you have been told to avoid it (placenta previa, bleeding, etc.).

Sports. Contact sports. Sports with fall risk (skiing, horseback riding, biking). Water sports. Scuba diving (most guidance says no during pregnancy, ask).

Hot activities. Hot tubs, saunas, steam rooms. Hot yoga. Hot baths (how hot is too hot). Heated workouts.

Low-impact. Walking guidance. Prenatal yoga. Swimming. Pilates.

Hobbies. Crafts with chemicals (resin, certain paints, soldering). Heavy gardening. Pottery (some glazes contain materials worth discussing).

Chapter 5: Environment

Household chemicals. Bleach and cleaning products. Bug sprays. Pesticides in the yard. Drain cleaners.

Paint and renovation. Painting the nursery. Sanding old (possibly lead) paint. Wallpaper removal. Asbestos (very old houses). Off-gassing from new furniture, mattresses, flooring.

Pets. Cat litter (toxoplasmosis is a known consideration). Cleaning pet waste. Pet medications you handle. Bringing a new pet home during pregnancy.

Salons and personal care. Hair dye and bleach. Nail salons (acrylics, gel, the chemical exposure). Spray tans. Botox and other cosmetic procedures. Whitening teeth. Acne medications (route to provider, name not in this book).

Electronics and radiation. Microwaves. Cell phones. Airport security scanners. Dental x-rays. Medical imaging (your provider will weigh).

Heat exposure. Heating pads (most are okay, ask). Electric blankets. Hot car or hot day exposure.

Outdoor environment. Air quality and wildfire smoke. High altitude. Sun exposure and sunscreen.

Chapter 6: Medications by Class

This book does NOT name medications. Your provider does, and your pharmacist confirms. The categories below are listed by purpose so you can identify what to bring to the conversation.

Pain and fever. "I have a headache" or "I have a fever," your provider has a specific recommendation. Do not assume what you took before is the same recommendation now. Bring the specific symptom, how often it happens, what you have been taking, and the dose and frequency.

Allergies. "I have seasonal allergies." Some options are routinely okayed in pregnancy, some are not. Ask before refilling your usual prescription.

Cold and flu. Multi-symptom cold medications often contain multiple ingredients, some of which are not first-line in pregnancy. Ask your provider for a recommendation BEFORE getting sick if possible. Get the flu shot conversation with your provider (a separate topic).

Gastrointestinal. Heartburn, constipation, diarrhea, nausea. Each has a different provider-recommendation conversation.

Mental health. If you take medication for depression, anxiety, ADHD, bipolar disorder, or any other mental-health condition, this is a conversation to have with both your prescriber AND your OB, ideally BEFORE pregnancy starts. The risk of untreated illness is not zero. The risk of any particular medication is also not zero. The two-doctor conversation balances them.

Chronic conditions. Asthma, diabetes, thyroid, high blood pressure, autoimmune conditions. Pregnancy changes how each is managed. Coordinate with both your specialist and your OB.

Vitamins and supplements. Prenatal vitamins are standard. Folate / folic acid is standard. Beyond that, every supplement deserves a conversation. Some are recommended (D, omega-3 in some cases, iron when low). Some are not (high-dose A is a known issue, certain herbs). Your provider has the current guidance.

Everything else. Sleep aids, antibiotics if you get an infection, antifungals, numbing creams, anything you put on your skin, anything you put in your mouth, all of these are conversations.

A reminder: Never assume the over-the-counter medication you took before pregnancy is the right choice now. The shelf at the pharmacy is not a pregnancy-vetted shelf. Ask before taking.

Chapter 7: Workplace

Physical demands. Lifting limits (your provider has specifics for you). Long stretches of standing. Long stretches of sitting. Climbing ladders or stairs at work. Night shifts and rotating shifts.

Environmental exposures. Chemicals you work with (chemists, lab workers, hairdressers, nail techs, agricultural workers, manufacturing). Radiation (medical workers, certain manufacturing). Heat exposure (kitchens, foundries, outdoor in summer). Biological hazards (healthcare, childcare, veterinary). Vibration and prolonged sitting (drivers, pilots).

Accommodations. Most places have a process for pregnancy-related workplace accommodations. The Pregnancy Workers Fairness Act in the US (and similar laws elsewhere) makes reasonable accommodations broadly available. Modified duties. Extra breaks. Sitting where you used to stand. A different shift. A reassignment from a specific exposure. Time off for appointments.

A note: A letter from your provider often helps get accommodations approved. Ask for one.

(For the deeper conversation templates with HR, your manager, and leave-planning details, see *The Pregnancy + Work Planner* on notes.soothemade.com.)

Chapter 8: Travel

Air travel. Until what week. Most airlines have policies (often 36 weeks domestic, earlier internationally). Compression socks. Hydration. Aisle vs window. Letter from provider (some airlines require this past 28 weeks).

Car travel. Long road trips (frequent stops to walk). Seatbelt positioning over the bump. Driving comfortably later in pregnancy.

Cruise. Cruise lines have policies (often won't board past 24 weeks). On-board medical care.

International. Vaccinations needed for the destination (some are not recommended in pregnancy). Zika and other disease considerations for the destination. Healthcare access at destination. Travel insurance with pregnancy coverage. Letter from provider.

Altitude. High-altitude destinations. Sudden altitude changes.

Chapter 9: "What They Say But Actually"

This chapter is for the kind of question that has a one-line internet answer that is misleading. The book's stance: ask your provider. Here are the typical reasons people get confused.

"Never X" in pregnancy. When you see "never X" in pregnancy advice, it usually means the RISK is non-zero and the BENEFIT is low. For most "never X" rules, asking your provider can clarify whether your specific risk is meaningful or not.

"X is fine" in pregnancy. When you see "X is fine," it usually means the risk is low at normal amounts. "Normal amounts" varies by person. Ask your provider what normal means for you.

"I heard on a podcast." Podcasts and influencer accounts are not your provider. They are sometimes accurate, sometimes not. They are usually generic. Your provider knows your specifics.

"Someone on a forum said." Forum posts are anecdotes. Your provider has evidence.

"My own mother did X in her pregnancy and I was fine." Pregnancy guidance has changed in many areas. Your mother's pregnancy followed different recommendations. Ask what's current.

"My friend's doctor said Y." Friend's doctor may be right for your friend. Your friend's pregnancy is not yours.

The pattern: whenever you encounter a strong claim about pregnancy safety online, write it down. Bring it to your provider. They will tell you whether it applies to you or not.

Chapter 10: "If I Already Did This Thing"

The internet often makes pregnant people feel like every past action is a catastrophe. It usually isn't. This chapter is the calm framework for the past-tense question.

You did the thing. Maybe before you knew you were pregnant. Maybe because the internet was wrong. Maybe because life was happening.

Here is what to do.

- 1. Write down what happened, with details.** What you did. How much. When (week of pregnancy). Anything else relevant.
- 2. Mention it at your next appointment, or call the nurse line if it feels urgent.**
- 3. Do NOT spiral on the internet between now and the appointment.**
- 4. Trust your provider's interpretation.**

A reminder: pregnancy is resilient. The baby is protected by many layers. A single exposure or accident or forgotten dose is rarely the disaster the internet will suggest. Providers have seen all of it. The provider is the right judge of your situation.

The opposite is also true. If you genuinely are worried about a specific exposure or event, call the provider. The nurse line is there for this. Calling does not make you neurotic. It makes you informed.

Chapter 11: Your Question List

The point of this whole book. Bring a list to your appointment.

Start a notes-app file titled "Pregnancy Questions." Every time you have a question between appointments, add it. Categorize by chapter if you want (food, drink, activity, etc.). When you arrive for your prenatal, hand the list to your provider, or just read it aloud.

Most providers prefer this. A patient with a written list is a patient who has thought about their care. The 15-minute appointment becomes useful because the patient has organized what to ask.

A few categories to consider in your list:

- **Top three questions.** The ones you most want answered, in case the appointment runs short.
- **Food and drink questions.**
- **Activity questions.**
- **Environment questions.**
- **Medication questions** (bring the symptom + what you have been considering, NOT a self-prescribed medication name).
- **Work questions.**
- **Travel questions.**
- **Anything that happened that you are worried about.**

After the appointment, write down what they said next to each question. Bring the list back next time.

Chapter 12: A Final Note

You will have a hundred questions in this pregnancy. Some will be small. Some will keep you up.

The book is the bridge. The provider is the answer. The two together make pregnancy meaningfully less anxious than the internet alone.

When you find yourself googling something at 11pm, write it down in the notes app instead. By Tuesday's appointment, you have a list. By the appointment after, you have answers.

You are not being difficult by asking. You are being a good advocate for the pregnancy. Good providers want patients who ask.

With care, Maya

About Soothemade

Soothemade is a small apothecary of printables, planners, cards, and books for new parents.

The printable companion to this book is the **Pregnancy Safety Reference** at notes.soothemade.com, with question-list template pages you can print and bring to appointments.

Also from Soothemade

- **The Pregnancy Week-by-Week Journal**, the data-heavy weekly companion.
 - **A Trimester-by-Trimester Emotional Companion**, the slow journal that tracks feelings rather than data.
 - **The Birth Anxiety Workbook**, for the specific anxiety the safety questions feed.
 - **The Pregnancy + Work Planner**, for the deeper accommodation + leave conversation.
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Crisis lines

For when the anxiety about safety becomes bigger than questions.

United States

- 988, Suicide and Crisis Lifeline
- 1-833-852-6262, Postpartum Support International (also serves prenatal)
- 1-833-943-5746, Maternal Mental Health Hotline
- 1-800-222-1222, Poison Control

United Kingdom

- 116 123, Samaritans

Canada

- 9-8-8, Suicide Crisis Helpline

Australia

- 1300 726 306, PANDA

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A Soothemade book. Made slowly, in plain language.